Ameritas Life Insurance Corp.

CRITICAL, CHRONIC AND TERMINAL ILLNESS ACCELERATED BENEFIT RIDER

TAX CONSEQUENCES. BENEFITS PAID UNDER THIS RIDER MAY BE TAXABLE. SOME OF THE BENEFITS PROVIDED BY THIS RIDER ARE INTENDED TO BE NON-TAXABLE. HOWEVER, CHANGES TO TAX LAW CAN CHANGE THE TAXABILITY OF BENEFITS. *WE* RECOMMEND THAT *YOU* CONTACT *YOUR* TAX ADVISOR TO ASSESS THE IMPACT OF THESE BENEFITS.

THE DEATH BENEFIT PAYABLE TO *YOUR* BENEFICIARY WILL BE REDUCED SHOULD *YOU* CHOOSE TO EXERCISE *YOUR* RIGHT UNDER THIS RIDER.

PURPOSE. This rider is attached to and made a part of this life insurance policy. If there is a conflict between the terms of the policy and the terms of this rider, the rider controls.

We will accelerate a portion of the policy's death benefit for a qualifying event subject to the provisions of this rider. The policy's death benefit will be reduced by the amount accelerated. The amount payable will be less than the amount accelerated.

QUALIFYING EVENTS. There are 17 conditions, classified under three illnesses, which constitute qualifying events covered under this rider.

Illness	Insured's Condition
Critical Illness	Invasive Life Threatening Cancer
	Stroke
	Major Heart Attack
	End-Stage Renal Failure
	Major Organ Transplant
	ALS (Amyotrophic Lateral Sclerosis)
	Blindness due to Diabetes
	Paralysis of two or more Limbs
	Major Burns
	Coma
	Aplastic Anemia
	Benign Brain Tumor
	Aortic Aneurysm
	Heart Valve Replacement
	Coronary Artery Bypass Graft Surgery
Chronic Illness	The inability to perform at least two *activities of daily living* or the presence of a *severe cognitive impairment*.
Terminal Illness	The presence of an illness or condition where life expectancy does not exceed 12 months.

If less than one year has passed since the initial acceleration of an event and the sum of all accelerated death benefits under this rider has not reached the maximum accelerated death benefit, then *you* are entitled to accelerate additional death benefits under this same event. The ratio of paid proceeds to the accelerated death benefit amount will remain unchanged for this annual period.

You are limited to five policy death benefit accelerations. This includes a first time event, a subsequent acceleration for the same event, and the reoccurrence of a previous event.

ACCELERATED DEATH BENEFIT. The maximum accelerated death benefit is shown on the policy schedule. The sum of all accelerated death benefits may not exceed the maximum accelerated death benefit shown on the policy schedule.

The amount payable associated with an accelerated death benefit will be paid in a lump sum for *critical illness* and *terminal illness* and may or may not be paid in a lump sum for *chronic illness*. If the amount payable under a *chronic illness* qualifying event does not exceed the per diem limitation declared each year by the Internal Revenue Service, then this amount payable will be in a lump sum. If the amount payable under a *chronic illness* qualifying event exceeds the per diem limitation declared each year by the Internal Revenue Service, then this amount payable will be in a lump sum or in installments, as chosen by *you*.

If *you* select to receive installment payments, then the first payment will be made immediately but will be constrained to the per diem limitation. A second payment will occur on the first business day of the calendar year following the initial payment. The amount of the second payment will be constrained to the lesser of the remaining amount owed or the per diem limitation for this new calendar year. Thereafter, if proceeds remain payable, they may be requested no more frequently than once per year, where the year commences on the anniversary date of the initial payment. Such requests will be subject to recertification that the *insured* continues to qualify for payment and, like the first two payments, may be split into one payment on the date of the request and one payment on the first day of the following calendar year so as to maintain that neither payment exceeds the per diem limit. Each payment will continue to be constrained to the lesser of the remaining amount owed or the per diem limitation. Each of the installment payments will need to be initiated by *you*. Any subsequent *chronic illness* payments will not be counted toward the policy limitation pertaining to the number of times the death benefit may be accelerated.

Payments anticipated after the initial payment could be adjusted or no longer applicable if *you* choose to accelerate under some other qualifying event. Any amount payable will be paid to *you* or *your* estate while the *insured* is living, unless the death benefit has been otherwise assigned or designated by *you*. The accelerated death benefit must be requested before the policy anniversary on which the *insured* attains the maximum acceleration age shown on the policy schedule.

Upon receiving a payment associated with an accelerated death benefit, *you* and any irrevocable beneficiary will be provided with a statement demonstrating the effect of the accelerated death benefit on the death benefit and premium. The basic policy premium, not including the policy fee, will be reduced by the same percentage as the percentage reduction in the policy's death benefit due to the accelerated death benefit.

If the *insured* dies before all payments associated with an accelerated death benefit are made, then future payments associated with the accelerated death benefit will not be made and the death benefit at this time will be paid pursuant to the policy. If the *insured* dies after *you* request the accelerated death benefit but before the associated payment is received, then the request shall be canceled and the death benefit, as if no accelerated death benefit request occurred, will be paid pursuant to the policy.

DETERMINATION OF AMOUNT PAYABLE. *We* will determine the amount payable for a given accelerated death benefit. Such payment is determined as follows:

(1) the accelerated death benefit; less
(2) the actuarial discount amount; less
(3) the administrative fee, as shown on the policy schedule; less
(4) any due and unpaid premiums.

The actuarial discount amount is the difference between (i) the accelerated death benefit and (ii) the present value of the net accelerated death benefit. The present value of the net accelerated death benefit is the difference between (i) the present value of the accelerated death benefit and (ii) the present value of the reduction in expected future premiums resulting from the reduction in the policy's death benefit.

The actuarial discount amount is determined based upon mortality rates and an interest rate. The mortality rates reflect the *insured's* health at the time of the death benefit acceleration. The interest rate will not exceed the greater of:

(1) the current yield on the ninety-day Treasury bill;
(2) the current maximum adjustable policy loan interest rate based on the Moody's Corporate Bond Yield Averages - Monthly Average Corporates published by Moody's Investor Service, Inc., or successor thereto, for the calendar month ending two months before the date of application for an accelerated payment; or
(3) 6%.

In the event that the Moody's Corporate Bond Yield Average - Monthly Average Corporates is no longer published, a substantially similar average, established by regulation issued by the Insurance Supervisory Official of the state in which the policy is delivered, will be used.

The present value of accelerated death benefits will always be less than the total accelerated death benefit requested due to discounting for mortality, interest, and future premiums.

CONDITIONS. Accelerated death benefits are subject to the following conditions:

(1) The *insured* must provide satisfactory proof that the *insured* has suffered a qualifying event. Satisfactory proof for a qualifying event must come from a *physician*. Such satisfactory proof must specify that the *insured* has been diagnosed as having a *critical illness*, *chronic illness*, or *terminal illness*.

(2) *We* reserve the right to obtain a second medical opinion at *our* expense. If *our physician's* medical opinion does not confirm the medical opinion *we* receive from the *insured's physician*, *we* may require, at *our* expense, an additional medical examination of the *insured* by a *physician* mutually acceptable to *you* and *us*. *We* will accept the medical opinion of this third *physician* as controlling.
(3) Any amount required to prevent the policy from terminating must be paid before any payment associated with an accelerated death benefit will be paid.
(4) Any existing assignee or lienholder must release their assignment or existing liens.
(5) Any irrevocable beneficiary must consent in writing to *your* request for the accelerated death benefit**.**
(6) *We* must receive a written claim form that will be provided by *us* within 10 working days after *you* request it. If the claim form is not provided within 10 days, it is considered that the claimant complied with the claim requirements if the claimant submits written proof covering the occurrence, the character, and the extent of the occurrence for which the claim is made. The associated payment of the accelerated death benefit is due immediately upon receipt of the due written proof of eligibility.
(7) The sum of accelerated death benefits on the *insured* from this policy will not exceed the maximum accelerated death benefit amount shown on the policy schedule.
(8) This rider allows for the accelerated payment of death benefit proceeds, which would otherwise be payable to the beneficiary of the policy. *You* are not eligible for this benefit if *you* will be required by:
   (a) law to use this benefit to meet the claims of creditors, whether in bankruptcy or otherwise; or
   (b) a government agency to use this benefit in order to apply for, obtain, or otherwise keep a government benefit or entitlement.
(9) If *you* are receiving a payment associated with a *chronic illness* accelerated death benefit, *we* may require certification by a *physician* once per calendar year that the *insured* has a *chronic illness*.

PREMIUMS DUE. Following any accelerated death benefit, *you* are required to pay the premium necessary to keep the base policy and any attached riders in force. Future policy premiums, not including the policy fee, will be reduced by the same percentage as the percentage reduction in the death benefit. The future policy fee remains unchanged.

EFFECT ON OTHER RIDERS. Optional rider benefits under this policy will continue to remain in force subject to the terms and conditions of the policy and riders. Charges for optional riders will be calculated according to the terms of the rider form. Trigger of any accelerated death benefit will have no effect upon any accidental death benefit riders or children's term life insurance riders.

GENERAL PROVISIONS

EFFECTIVE DATE. The effective date of this rider is shown on the policy schedule.

REINSTATEMENT. If the policy lapses and is reinstated, this rider will be reinstated with the policy.

TERMINATION. This rider will end at the earlier of written request from *you* or termination of the policy. If this rider is terminated, this shall not prejudice the payment of benefits for any qualifying event that occurred while the form was in force.

DEFINITIONS

ACTIVITIES OF DAILY LIVING.

(1) Dressing: the ability to put on and take off all garments and medically necessary braces or artificial limbs usually worn, and to fasten or unfasten them.
(2) Toileting: the ability, with or without the help of adaptive devices, to get to and from or on and off the toilet; and to perform associated personal hygiene.
(3) Transferring: the ability to move in and out of a chair, bed, or wheelchair with or without equipment such as canes, quad canes, walkers, crutches or grab bars or other support devices including mechanical or motorized devices.
(4) Continence: the ability to voluntarily control bowel and bladder function, or, in the event of incontinence, the ability to maintain a reasonable level of personal hygiene including caring for a catheter or colostomy bag.
(5) Eating: the ability to feed yourself by getting food into *your* body from a receptacle (such as a plate or cup or table) or by a feeding tube or intravenously.
(6) Bathing: the ability to wash yourself, with or without the help of adaptive devices, by sponge bath; or in the tub or shower, including the task of getting in and out of the tub or shower.

CHRONIC ILLNESS. The *insured*:

(1) is unable to perform (without hands-on assistance) at least two *activities of daily living*, and has been unable to perform them for a period of at least 90 days; or
(2) has suffered *severe cognitive impairment* to the extent that *substantial supervision* is required to ensure the *insured's* health and safety.

CRITICAL ILLNESS. One of the following events experienced by the *insured*:

Condition	Description
Invasive Life Threatening Cancer	A definite diagnosis of a disease manifested by the presence of one or more malignant tumors and characterized by the uncontrolled growth and spread of malignant cells and the invasion of normal neighboring tissue or distant lymph node or organ metastasis. Types of cancer include carcinoma, melanoma, leukemia, lymphoma, multiple myeloma, myelodysplastic syndrome, and sarcoma. The diagnosis of Invasive Life Threatening Cancer must be established according to the criteria of malignancy established by a board certified specialist acting with their specialty The American Board of Pathology after a study of the histocytologic architecture or pattern of the suspect tumor, tissue, or specimen that confirms such malignancy. The date of the histopathological examination that establishes a definite diagnosis. No benefit will be payable under this condition for any of the following: (a) Pre-malignant lesions, benign tumors, polyps, or dysplasia; (b) Carcinoma in-situ; (c) Any non-melanoma skin cancer, except those with distant lymph node or organ metastasis; (d) Malignant melanoma skin cancer that is less than or equal to 1.0 mm in thickness, unless it is ulcerated or is accompanied by distant lymph node or organ metastasis. (e) chronic lymphocytic leukemia classified as Rai Stage 0. (f) early prostate cancer diagnosed as T1a or T1b by the AJCC Staging System without distant lymph node or metastasis. (g) papillary thyroid cancer or follicular thyroid cancer, or both, that is less than or equal to 2 cm in greatest diameter and is classified as T1 by the AJCC Staging System, without lymph node or distant metastasis.

Stroke	A cerebrovascular incident caused by infarction or brain tissue, cerebral hemorrhage, thrombosis or embolization from an extra-cranial source and producing a measurable neurological deficit that persists for at least 30 consecutive days following the occurrence of the stroke. Stroke does NOT include Transient Ischemic Attacks (TIAs), Vertebro-basilar insufficiency, retinal vessel illnesses, lacunar infarcts which do not meet the definition of stroke as described above or incidental findings on neuroimaging studies. The diagnosis of stroke must be made by a neurologist based on documented neurological deficits and confirmatory neuroimaging studies. Intracerebral vascular events due to trauma are not covered.
Major Heart Attack	The death of a portion of the heart muscle resulting from obstruction of blood supply to the relevant area. Major Heart Attack does NOT include angina or the chance finding of electrocardiographic (EKG) changes indicative of a previous heart attack. The diagnosis of Major Heart Attack must be made by a *physician* and be based on the presence of a diagnostic elevation of cardiac enzymes or biomedical markers and the presence of chest pain and at least one of the following: (a) new electrocardiographic (EKG) changes which support the diagnosis, or (b) identification of an intracoronary thrombus by angiography, or (c) imaging evidence of a new loss of viable heart muscle or a new regional wall motion abnormality.
End Stage Renal Failure	The irreversible and total failure of both kidneys in which the use of hemodialysis or peritoneal dialysis is deemed to be medically necessary. The diagnosis must be established by a Consultant Nephrologist.
Major Organ Transplant	A definite diagnosis of the irreversible failure of any of the following organs or tissues: (a) heart; (b) both lungs; (c) liver; (d) both kidneys; (e) pancreas; (f) bone marrow. Transplantation must be medically necessary, and must be documented as such by a Transplant specialist. The *insured* must be placed on a transplant list or have been the recipient of a heart, lungs, liver, kidneys, pancreas or bone marrow, and limited to these entities.
ALS	A definite diagnosis of ALS (Amyotrophic Lateral Sclerosis) diagnosed by a Consultant Neurologist.
Blindness due to Diabetes	A definite diagnosis of the total and irreversible loss of vision in both eyes solely as a result of diabetic retinopathy. The diagnosis of blindness must be made by a Consultant Ophthalmologist and be evidence by: (a) the corrected visual acuity being 20/200 or less in both eyes; or (b) the field of vision being less than 20 degrees in both eyes.
Paralysis of two or more Limbs	The complete and permanent loss of use of two or more limbs through neurological injury for a continuous period of at least 90 days. Paralysis must be confirmed by a *physician* board certified in Neurology.
Major Burns	The definite diagnosis of burns that are a full-thickness or third-degree burn covering at least 20% of the body surface. The diagnosis must be established by a hospital unit.

Coma	A profound state of unconsciousness from which the *insured* cannot be aroused to consciousness, and in which stimulation will produce no more than primitive avoidance reflexes, which last for a period of at least 96 hours, and for which period the Glasgow coma score must be 4 or less. A definite diagnosis of coma must be documented by evidence of a neurological deficit that is expected to last for a continuous 12-month period or longer from the date of the diagnosis to determine coma. Life support systems must be required throughout the period of unconsciousness as well as the following exclusions: (1) Coma secondary to any alcohol or drug abuse and/or narcotics are not covered by this definition. (2) Coma caused/prolonged due to therapeutic reasons is not included in this definition.
Aplastic Anemia	A definite diagnosis of a chronic persistent bone marrow failure, confirmed by biopsy, which results in anemia, neutropenia and thrombocytopenia requiring blood product transfusion, and treatment with at least one of the following: (a) Marrow stimulating agents; (b) Immunosuppressive agents; (c) Bone marrow transplantation. The diagnosis of Aplastic Anemia must be made by a specialist.
Benign Brain Tumor	A non-cancerous tumor within the cerebral tissue or the cerebral meninges resulting in permanent neurological symptoms and where open, surgical intervention is deemed medically necessary. The tumor must be confirmed by a CT scan or MRI examination of the brain. Tumors in the pituitary gland, cysts, granulomas and tumors in the cranial nerves (e.g. acoustic neuroma), or malformations in, or of, the brain substance, cerebral arteries or veins and/or the spinal cord are not covered by this definition.
Aortic Aneurysm	A definite diagnosis by a specialist that intervention is deemed medically necessary for disease or trauma to the aorta requiring either an open surgical repair with excision and replacement of the diseased or traumatized aorta with a graft or an endovascular repair with a stent graft. Aorta refers to the thoracic and abdominal aorta but not its branches.
Heart Valve Replacement	A definite diagnosis determined by a specialist that surgery is medically necessary to replace any heart valve with either a natural or mechanical valve. Heart valve surgery or repair utilizing transvascular percutaneous valve procedures are not covered by this defintion.
Coronary Artery Bypass Graft Surgery	A definite diagnosis by a specialist that surgery is medically necessary to correct narrowing or blockage of one or more coronary arteries with bypass grafts(s). Any other surgical procedure, such as but not limtied to, angioplasty, intra-arterial procedures, or trans-catheter percutaneous procedures are not covered.

PHYSICIAN. A doctor of medicine or osteopathy (other than *you*, *your* spouse, domestic partner, a member of *your* family, a business or professional partner, or any person with whom *you* share a financial or business interest) licensed to practice medicine and surgery in the state in which he or she practices and who is practicing within the scope of such license in the United States.

SEVERE COGNITIVE IMPAIRMENT. A diagnosis by a *physician* who is a certified neurologist of Alzheimer's disease and similar forms of irreversible dementia supported by medical evidence of a loss or deterioration in intellectual capacity that includes (1) short-or long-term memory, (2) orientation to people, places, or time, (3) abstract or deductive reasoning, and (4) judgment as it pertains to safety.

SUBSTANTIAL SUPERVISION. Continual, 24-hour supervision by another person that is necessary to protect severely cognitively-impaired individuals against threats to their health or safety.

TERMINAL ILLNESS. The *insured* has been certified by a *physician* as having an illness or physical condition which can reasonably be expected to result in death within 12 months after the date of certification.

In the event *you* need to contact someone about this rider for any reason, please contact *your* agent. If *you* have additional questions, *you* may contact Ameritas Life Insurance Corp. at [PO Box 81889, Lincoln, NE 68501, 1-800-745-1112].

AMERITAS LIFE INSURANCE CORP.

President

Secretary